Riley Physicians
Indiana University Health

DEVELOPMENTAL MEDICINE
PSYCHOLOGICAL EVALUATION

NAME: Peyton Reid
MRN: 74813166
PARENT:
Jessica Reid
908 West Neely Avenue
Muncie, IN 47302

DOE: 1/31/2022
DOB: 2/6/2015
CA: 6 years, 11 months, 25 days

SOURCE/REASON FOR REFERRAL: Eric Westergren, DO
Peyton is a nearly seven-year-old child who was referred by Dr. Eric Westergren, DO, due to parent concerns for possible autism spectrum disorder. Peyton was initially seen by psychologist Dr. Steven Koch, PhD, in the Division of Developmental Medicine for a diagnostic consultation on September 7, 2021. Peyton was referred for follow-up assessment to determine if his behavior meets diagnostic criteria for autism.

PROCEDURES ADMINISTERED:
Psychological Interview & Assessment
Adaptive Behavior Assessment System, Third Edition (ABAS-3)
Behavior Assessment System for Children, Third Edition, Parent Rating Scales (BASC-3)
Wechsler Intelligence Scale for Children, Fifth Edition (WISC-V)
Semi-Structured Play Assessment using tasks from the *Autism Diagnostic Observation Schedule, Second Edition, Module 3 (ADOS-2)*
Formal and informal observations
File review

Due to the current COVID-19 pandemic, accommodations were made to ensure the safety of Peyton, his parents, and the evaluation team. All procedures were developed in accordance with Indiana University Health – Riley Hospital for Children and Indiana University School of Medicine, Department of Pediatrics COVID-19 guidelines and regulations. Specific accommodations include the following:

- All face-to-face interaction was completed with required PPE (including use of masks) for Peyton, his parents, and the evaluation team.

Accommodations specific to each test, as well as how these accommodations may impact interpretation of results, are described in the test results (Findings Section) below.

BACKGROUND INFORMATION:

Developmental, medical, educational, and social history was originally provided by Peyton's parents during the diagnostic appointment on September 7, 2021, with Dr. Koch, and updated at today's appointment. Information from both appointments is integrated below.

Peyton lives in Muncie, Indiana, with his parents, two older brothers and younger sister. He is close with his paternal grandparents, and spends Fridays at their home.

Peyton represented his mother's third pregnancy. She noted that the pregnancy was complicated by intrauterine growth retardation. Complications and exposure to alcohol, tobacco, and other drugs during pregnancy were denied. Mrs. Reid also shared that she was later diagnosed with pericardium cardiomyopathy which started during the pregnancy and was identified after delivery. Peyton was delivered nearly four weeks early, weighing 4 pounds, 4 ounces. Mrs. Reid was hospitalized after delivery due to a collapsed lung. Peyton was transported to Riley Hospital due to body tremors which the hospital staff were concerned may have been seizures. After two weeks of observations, Peyton was discharged home. His parents described his early temperament as colicky. No early concerns with sleeping or eating as an infant were reported.

Peyton's caregivers were asked about early developmental milestones, which he reportedly met at the expected ages. Peyton first walked independently at around 13 months. He was toilet trained for urine at 18 months, and remains dry during the day and night now. Peyton will not defecate in a toilet, but continues to use a diaper for this. He will let his parents know once he has defecated so that he can be changed. Peyton's caregivers reported the following gross / fine motor current concerns. He requires assistance with dressing. His parents select his clothing, he requires help with putting on his socks / shoes, and with clothing fasteners. Peyton's mother shared that she also has to put his clean clothing in the dryer each morning to warm them up; otherwise Peyton will not put on his clothes if they are not warm. This is not an issue when he spends the night at his grandparent's home. He requires assistance with wiping after a bowel movement, and struggles to brush his teeth (he will gag when brushing). Peyton does not like to have his fingernails clipped. He is independent with using a spoon and fork.

Regarding language development, Mrs. Reid shared that Peyton spoke later than his siblings but was still within the expected range. He spoke single words at around 9-10 months of age, but then stopped speaking after his first birthday until around 18 months of age. At 18 months, Peyton began to speak immediately in full sentences. He would use early sign language to communicate needs. Peyton also would nod his head to indicate yes/no to questions asked. He would wave goodbye, but only after being prompted to do so. His caregivers reported the following present language skills. Peyton talks more like an adult. His parents noted that they often use more advanced language when talking to Peyton than what would be expected when talking with a six-year-old child. Peyton seems to understand the meaning behind jokes, but will tell others that the statement is not funny. He reportedly takes what others say literally and struggles with understanding figurative speech.

Prior medical history includes surgical correction of strabismus in October 2016. He currently wears glasses as needed for reading. Peyton attended a few weeks of preschool, but then refused to go. He felt he didn't need to go as school was boring. Peyton is currently takes Zyrtec (2.5mg daily). Mrs. Reid shared about a recent incident at school where Peyton passed out during the school day (during mass). In talking with Peyton's physician after the incident, Dr. Westergren felt that his episode was the result of a vagal response. He has experienced no similar incidents.

The family history is significant for autism spectrum disorder (maternal cousin), Attention Deficit Hyperactivity Disorder (father, oldest brother, paternal uncle), speech-language delays (oldest brother, younger sister), anxiety / depression (mother, father), neurofibromatosis / NF1 (paternal grandfather), and Grave's disease / hyperthyroidism (mother).

Peyton attends the first grade at St. Michael's Catholic Grade School. While he is doing well academically, his kindergarten teacher last year voiced concern with his social interactions. She shared that Peyton would spend his recess talking with the teacher about electronics instead of playing with his peers. Peyton described school as "okay." He shared that likes computers the most, and dislikes his "morning work."

Peyton's caregivers were interviewed to gain further information about his social-communication skills, social relationships and play, restricted interests, and repetitive behaviors. In terms of social-communication skill development, Peyton would raise his hands in anticipation of being picked up; however, he often did not want to be carried. He also did not like hugs or kisses by his mother as a toddler; he was more affectionate towards his grandmother.

Peyton would reach (vs. point) to items to let his caregivers know what he wanted. He would also lead a parent by the hand when the item was in another room. He often shows emotions on his face as extremes. His caregivers noted that they often cannot tell how he is feeling with more subtle emotions. Peyton may recognize emotions in others, but can struggle with understanding why they are showing the emotions they have. His parents shared that at a family funeral, Peyton was asking why everyone was sad. Peyton may ask for help when he cannot complete a task independently. He will also get angry and throw the item if unable to do what is trying to do.

Eye contact is reported as being poor when he was younger. At present, he has gotten better with eye contact, but it is still described as "not great. His parents shared that he often will look to the side or close his eyes when talking with someone. Peyton shared that it can be uncomfortable to look directly at someone.

Peyton used to play with Paw Patrol toys when he was younger, as well as playing with a train set by putting the train track together. When playing with the Paw Patrol toys, he would create his own stories with the characters. He did not engage in as much pretend play as his brothers, but would at times do so (e.g., playing McDonalds where he would take orders from others). He

has a favorite stuffed bear, which he brought with him to today's appointment. Peyton stated that he has had this bear since he was a baby.

Peyton will engage in talking with others; his parents shared that the conversations often are one-sided more than a back-and-forth discussion. If he gets interrupted in the middle of sharing information with a parent, he will restart his story from the beginning (vs. picking up where he left off on the conversation) once he can again speak. He has great difficulty making a choice when asked to choose between two things (such as being asked if he wants milk or juice).

With regard to peer interactions, Peyton's parents described him as a "loner," someone who often will play by himself. They stated that he has one friend, a girl who has similar behaviors to Peyton. When he does play with other children, his parents and grandmother noted that he tends to play more alongside others than playing interactively. He does not often initiate play with peers. Peyton will join in play with others. Peyton stated that others do not ask him to play with them. His parents noted that when other children do ask him to play, Peyton will often respond by saying, "no thank you." He can be shy around strangers. His mother indicated no current concerns with teasing, but voiced having concern in the future from peer teasing.

Regarding restricted interests and repetitive behaviors, Peyton's caregivers that does engage in flapping and jumping up and down. He likes to go to Walmart where he will walk up and down the electronics aisle looking at the items while talking to himself and flapping his hands. He used to toe walk when younger but does not do so at present. When he is sad or hurt, Peyton will hold his breath. He has held it to the point that he has passed out in the past. His caregivers denied Peyton showing a fascination with watching items spinning. He often will line up his cars or blocks. Peyton's mother noted that he has an established routine where he must have his clothes come out of the clothes dryer before he will put them on. Peyton has a particular interest in talking and learning about sweepers, microwaves, iPhones and Windows operating systems. He has learned everything he can about every iPhone model made and every version of Windows. He watches YouTube videos of how to fix computers, and has taken apart and fixed computers. Peyton will have a meltdown with changes in the expected routine. His meltdowns include him screaming, hitting others, and throwing items.

Peyton eats well but will refuse to try new foods. He shared that he does not like how foods taste in his mouth. He will put his hands over his ears when he hears certain sounds, which has included vacuums, automatic hand dryers, and fireworks. He prefers to wear soft clothing fabrics. His parents shared that he has a high pain tolerance, but will hold his breath when he is hurt. He will help his grandmother with cooking, but does not like it when he gets food on his hands (e.g., when cracking eggs while making cookies). His mother shared that she recently started using a weighted blanket with Peyton, which has helped to calm him. Peyton likes to take care of the family's cats by picking them up an carrying them around the house. His parents shared that the cats are tolerant of Peyton carrying them around. He sleeps well once he falls asleep; he can have difficulty falling asleep at times.

Peyton's behavior was also observed during the diagnostic appointment in September 2021. Peyton held his toy bear off and on during the interview. At times, he held it by the bear's nightcap, while swinging the bear back and forth in front of him. Eye contact with the examiner and his parents was poor. At first glance, he appeared to be looking at others when talking with them but Peyton often had his eyes closed while talking. Peyton would correct others at how they stated things during conversation. For example, when the examiner and his caregivers were discussing Peyton's attending preschool, he corrected his mother by stating, "it wasn't preschool. It was 'pre-K'." At another time, his mother shared that Peyton still uses his pacifier, and Peyton corrected her, saying it was his "pacie." He often talked while using his hands for emphasis. He would share information about his areas of interest, at times interrupting the conversation to interject his comments. Peyton asked the examiner about the first Windows operating system that he used. When the examiner told him he used Windows version 3.2, Peyton explained that the operating system was released in 1993. Peyton moved about and fidgeted in his seat. At one point, he started to pace in circles in the room.

FINDINGS:

A total of 6.0 hours was spent in review of his medical record, test administration, interviews with Peyton and his parents, interpretation of test results, preparation of recommendations, face-to-face interpretive feedback, and in preparing this report.

Wechsler Intelligence Scale for Children, Fifth Edition (WISC-V)

Due to COVID-19 precautions described above, the results of the *Wechsler Intelligence Scale for Children, Fifth Edition (WISC-V)* should be interpreted with caution. The *WISC-V* was administered digitally while patient and examiner were in the same room; the reliability and validity of tablet administration and equivalency of tablet and paper/pencil administration have been demonstrated and accepted (e.g., Daniel, Wahlstrom, & Zhang, 2014). However, several elements, including face coverings, a plexiglass barrier, and increased distance between examiner and patient, introduce variables that have an unknown impact on the validity of the results. Consequently, the following results should be interpreted with caution. Nevertheless, this information provides valuable guidance regarding relative strengths and challenges and will be effective baseline estimates for future evaluation.

Peyton was administered the *Wechsler Intelligence Scale for Children – Fifth Edition*, a measure of general intellectual functioning. Results provide subtest and composite scores that represent intellectual abilities and skills in specific cognitive domains: Verbal Comprehension, Visual Spatial, Fluid Reasoning, Working Memory, and Processing Speed. Scores between 85 and 115 are considered to be average when compared with others his age.

Peyton obtained a Full-Scale IQ of 109, which falls within the average range of cognitive ability. This performance suggests Peyton may learn at a similar rate compared to other children his age. Autism preventing Peyton from demonstrating true IQ

The Verbal Comprehension Index measures both verbal comprehension and expression. This index measured Peyton's ability in word knowledge and verbal reasoning. Peyton performed in the high average range (Standard Score = 113). Peyton was able to form associations between words and define words.

The Visual-Spatial Index measures a child's ability to find visual patterns. Peyton performed in the average range (Standard Score = 100). Peyton was able to examine pieces of a visual "puzzle" and identify how they go together to make a larger image.

The Fluid Reasoning Index measures one's ability to solve novel problems using inductive or deductive reasoning. Peyton performed in the average range (Standard Score = 100). Peyton was able to identify visual patterns and relationships on items.

The Working Memory Index measures auditory and visual abilities in sequencing and mental manipulation. Peyton performed in the average range (Standard Score = 97). Peyton was able to recall information in the short-term when compared to his same-aged peers.

The Processing Speed Index measures an individual's speed in completing mental operations and psychomotor tasks, and requires visual attention, visual-motor coordination, and short-term visual memory in order to correctly scan and reproduce shapes. For these tasks, Peyton performed in the average range (Standard Score = 103).

Adaptive Behavior Assessment System, Third Edition (ABAS-3)
Mrs. Reid was asked to complete the *Adaptive Behavior Assessment System – Third Edition* checklist to assess Peyton's ability to perform everyday tasks at home and in the community. The General Adaptive Composite (GAC) summarizes performance across all adaptive skill areas. Peyton obtained a GAC score of 86. Peyton's current overall level of adaptive behavior is in the below average range, as high as or higher than 18% of individuals of the same age. Because the GAC provides the most complete measure of adaptive behavior, it is likely to be the most reliable and accurate estimate of overall adaptive functioning. However, more detailed information about Peyton's unique profile of adaptive functioning may be obtained by reviewing performance within adaptive domains and skill areas if significant differences exist between adaptive domain standard scores or skill area scaled scores.

The Conceptual domain standard score summarizes performance across the Communication, Functional Academics, and Self-Direction skill areas. Peyton's Conceptual domain standard score of 93 is in the average range, as high as or higher than 32% of individuals of the same age.

The Social domain standard score summarizes performance across the Leisure and Social skill areas. Peyton's Social domain standard score of 84 is in the below average range, as high as or higher than 14% of individuals of the same age.

The Practical domain standard score summarizes performance across the Community Use, Home Living, Health and Safety, and Self-Care skill areas. Peyton's Practical domain standard score of 85 is in the below average range, as high as or higher than 16% of individuals of the same age.

Behavior Assessment System for Children, Third Edition, Parent Rating Scales (BASC-3)

The *Behavior Assessment System for Children, Third Edition, Parent Rating Scales (BASC-3)* was completed by Mrs. Reid. This was used to assess behavioral concerns, such as hyperactivity, anxiousness, or depressed mood. Mrs. Reid indicated clinically significant concerns in the areas of aggressive behavior, conduct problems, and withdrawn behavior. She reported that Peyton almost always threatens to hurt others, throws or breaks items when angry, gets into trouble, often lies, prefers to play alone and is shy around other children. Mrs. Reid also noted at-risk levels of concerns in the areas of hyperactivity. She reported that Peyton almost always acts without thinking.

In the area of adaptive skills, Mrs. Reid indicated clinically significant concerns in the areas of activities of daily living and adaptability and at-risk levels of concern in the areas of social skills. Specifically, Mrs. Reid noted Peyton almost always has trouble following regular routines, had difficulty adjusting to changes, and does not help others to be their best.

Semi-structured Play Assessment using tasks from the *Autism Diagnostic Observation Schedule, Second Edition (ADOS-2)*

Due to COVID-19 precautions described above, a valid *Autism Diagnostic Observation Schedule – Second Edition* (*ADOS-2*) could not be administered. Instead, a semi-structured assessment using tasks from the ADOS-2 was conducted. The following tasks were used / modified because of the need for the child and examiner to wear masks: Construction task, Make-Believe Play, Joint Interactive Play, Demonstration Task, Description of a Picture, Telling Story from a Book, Cartoons, Conversation and Reporting, Emotions, Social Difficulties and Annoyances, Friends / Relationships / Marriage, and Loneliness.

While the tasks from the *ADOS-2* provides information about possible autism spectrum disorders, the instrument does not provide a diagnosis. Instead, a rating is obtained that provides an overview of the child's performance and is viewed in context with other information. Tasks from Module 3 were administered due to Peyton's age and communication skills. The following provides a summary of the clinical observations:

During make-believe and joint interactive play, Peyton played briefly with the toys before stating, "I don't know what else to do with these toys. Do you have Legos?" During joint interactive play, Peyton informed the examiner that he could play the "bad guy," and directed the examiner as to how to interact by stating, "but he doesn't talk." While reading a book together, Peyton corrected the examiner's selection of the starting page for the book, informing him that "that's the title page." The story explored how frogs were flying around at night, and lost their ability to fly once the sun came up. Peyton appeared confused with the premise to the story and asked the examiner, "why would that [sunrise] make them stop flying?"

Confused or asking for clarification?
need to have toys. He is interested in playing
Mario, Luigi, Peach & Bowser McDonald's - PL
both toys at

[redacted]ng activities and conversation with the examiner, Peyton presented with a flat affect and was not able to identify emotions or feelings in characters of stories or pictures. When asked to describe his own feelings, Peyton would often provide simple, brief responses or state he could not explain. For example, when asked how he feels when he is happy, Peyton replied, "I can't really [explain]. I'm just happy." He became more animated with expressing emotions when talking about computers. He shared that he was "sooo excited" when he got his computer, explaining to the examiner that it had "30 gigs left" and informed the examiner that the computer was made in 2011. He then explained how he "downgraded" the computer to Windows 7.

DIAGNOSIS/IMPRESSIONS:

Peyton is a nearly seven-year-old boy who demonstrates persistent deficits in social communication (i.e., social emotional reciprocity; integration of verbal and nonverbal communicative behaviors; developing and maintaining friendships) as well as the presence of restricted and repetitive behaviors (i.e., stereotyped motor movements; use of objects and speech; inflexible adherence to routines and ritualized behaviors; fixated interests that are unusual in intensity/focus; hyper/hypo-reactivity to sensory stimuli). It is my impression that Peyton's behaviors meet the diagnostic criteria for a diagnosis of **Autism Spectrum Disorder (DSM-5 299.0; ICD-10 F84.0)**.

RECOMMENDATIONS:

Written recommendations were provided on the day of the appointment.

1. It is recommended that my full report (to be sent in a few weeks) be shared with Peyton's school so that appropriate educational placement and planning may occur. The school should consider providing special education services under the category of Autism Spectrum Disorder.
2. Peyton may benefit from individual counseling, using a cognitive-behavioral therapy approach to help increase his social skills and emotion identification. Peyton's behavioral intervention providers may consider incorporating elements from the following curricula to address social communication and self-regulation skills:
    a. Skillstreaming resources by Arnold Goldstein and Ellen McGinnis (www.skillstreaming.com)
    b. Social Thinking resources by Michelle Garcia Winner (www.socialthinking.com)
    c. Social Skills Training for Children and Adolescents with Asperger's Syndrome and Social-Communication Problems by Jed Baker (2005, Autism Asperger Publishing Co.)
    d. The Gray Center for Social Learning and Understanding (www.thegraycenter.org and related books by C. Gray)
3. The following resources may be helpful for Peyton and his family:
    e. Healthcare Financing Resources

Family Voices Indiana: Family-led organization that provides information, education, training, outreach, and peer support to families of children and youth with special health care needs. Family Voices is an excellent resource for assistance related to healthcare financing. Phone: (317) 944-8982 / Website: www.fvindiana.org

Indiana Medicaid Waiver Program: Program through Bureau of Developmental Disabilities permits Medicaid to fund services such as speech, behavioral and other individualized supports in the home and community. Phone: (800) 545-7763 / Website: http://www.in.gov/fssa/ddrs/2639.htm

f. Educational Support

IN*Source: Parent advocacy group that assists with special education and educational planning. Phone: (800) 332-4433 / Website: www.insource.org

g. Resources for Information and Advocacy related to Autism Spectrum Disorder

Autism Speaks: Organization that provides information regarding Autism Spectrum Disorder as well as "Tool Kits" to assist caregivers, educators and professionals. Phone: (888) 288-4762 / Website: www.autismspeaks.org

Autism Society of Indiana: Offers services and support to individuals affected by Autism Spectrum Disorder and their families. Phone: (800) 609-8449 / Website: http://www.autismsocietyofindiana.org.

Autism Society: Organization that provides leadership in serving people on the autism spectrum, their families, and professionals. Phone: (800) 3AUTISM / Website: http://www.autism-society.org/

Indiana Resource Center for Autism: Organization focused on early detection and diagnosis, effective early childhood and school-age programs that utilize evidence-based practices, transition, post-secondary opportunities, community employment and living options, family support, and other activities that lead to an improved quality of life. Phone: 1-800-825-4733 Website: http://www.iidc.indiana.edu/pages/irca

4. Talk with Dr. Westergren about a physician's order for the following:
   a. An occupational therapy evaluation and treatment of fine motor and sensory difficulties
   b. A speech-language therapy evaluation and treatment of receptive, expressive and pragmatic language skills.
5. Due to difficulties with peer interactions, look for opportunities to involve Peyton in structured, adult-supervised peer interactions, such as scouts, church youth group, 4-H, or summer day camp.
6. The following resources may be helpful to the family in understanding the diagnosis and treatment:
   a. *100 Day Kit by Autism Speaks.* Website: *http://www.autismspeaks.org/family-services/tool-kits/100-day-kit*
   b. *Autism Spectrum Disorders: What Every Parent Needs to Know.* American Academy of Pediatrics. Edited by Alan I. Rosenblatt, M.D., FAAP, and Paul S. Carbone, M.D., FAAP (2012)

7. Because genetic causes can sometimes be identified for individuals with autism spectrum and other developmental disorders, the American Academy of Pediatrics now recommends that genetic studies, including fragile X and chromosome microarray, be completed when these diagnoses are confirmed. Please consult De. Westergren to determine if Peyton should have these or other studies and if a referral to genetics is needed.
8. I will refer the family to our genetics counselor in Developmental Medicine, to talk with parents more about questions they may have.

It was a pleasure to meet Peyton and his parents. Please do not hesitate to contact the Division of Developmental Medicine at (317) 944-4846 with any questions or concerns about this appointment.

Steven M. Koch, PhD, HSPP
Clinical Psychologist
Associate Professor of Clinical Pediatrics

CC: *Eric Westergren, DO*

ADDENDUM

WECHSLER INTELLIGENCE SCALE FOR CHILDREN – FIFTH EDITION

The Wechsler is an individual test of verbal and non-verbal intellectual functioning for children between the ages of 6-0 and 16-11.

Subtest	Scaled Score*
Block Design	11
Similarities	13
Matrix Reasoning	10
Digit Span	13
Coding	10
Vocabulary	12
Figure Weights	10
Visual Puzzles	9
Picture Span	6
Symbol Search	11

Index	Standard Score**
Verbal Comprehension	113
Visual Spatial	100
Fluid Reasoning	100
Working Memory	97
Processing Speed	103
Full Scale IQ	109

ADAPTIVE BEHAVIOR ASSESSMENT SYSTEM – THIRD EDITION

The Adaptive Behavior Assessment System provides a comprehensive, norm-references assessment of the adaptive skills of individuals who are school-aged to adulthood.

Adaptive Skill Area	Scaled Score*
Communication	8
Community Use	10
Functional Academics	11
Home Living	9
Health and Safety	7
Leisure	7
Self-Care	4
Self-Direction	7
Social	7
Composite	**Standard Score****
Conceptual	93
Social	84
Practical	85
General Adaptive Composite	86

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* Scores between 7 and 13 are considered average.

** Scores between 85 and 115 are considered average.

BEHAVIOR ASSESSMENT SYSTEM FOR CHILDREN – THIRD EDITION, PARENT RATING

The Behavior Assessment System for Children is a multidimensional approach to evaluate the behavior and self-perceptions of children. The child's caregiver completes the Parent Rating Scales.

Validity Scale	Rating
F Index	Acceptable
Response Pattern	Acceptable
Consistency	Acceptable
Clinical Scale	T-Score***
Hyperactivity	67
Aggression	84
Conduct Problems	76
Anxiety	68
Depression	62
Somatization	50
Attention Problems	61
Atypicality	64
Withdrawal	71
Adaptive Scale	
Adaptability	29
Social Skills	34
Leadership	53
Functional Communication	54
Activities of Daily Living	20
Clinical Indexes	
ADHD Probability	68
Autism Probability	69
Emotional-Behavioral Probability	74
Executive Impairment	67
Composite Scale	
Externalizing Problems	78
Internalizing Problems	62
Behavior Symptoms Index	73
Adaptive Skills	36

*T-Scores between 66 and 69 and between 31 and 35 are considered At-Risk. T-Scores above 70 and below 30 are considered Clinically Significant.

AUTISM SPECTRUM DISORDER: DSM-5 CHECKLIST

The ASD Checklist lists the Diagnostic and Statistical Manual of Mental Disorder's (DSM-5) current criteria related to a diagnosis of ASD. Examples of Peyton's behaviors observed / reported are indicated for each criterion.

Deficits in Social Communication (All)	**Example/Description**
Deficits in social-emotional reciprocity	Poor back-and-forth conversation
Deficits in nonverbal communication used for social interaction	Poorly integrated eye contact with verbal communication
Deficits in developing, maintaining, understanding relationships	Limited peer relationships, difficulties in emotion identification

Restricted, Repetitive and Stereotyped Patterns (2 of 4)	**Example/Description**
Stereotyped / repetitive motor movements	Parent reports of flapping, hand mannerisms
Insistence on sameness, inflexible adherence to routines	Difficulties adjusting to changes in expected routine
Highly restricted, fixated interests	Intense interests in computer operating systems, video games
Hyper- or hypo-reactivity to sensory input	Sound sensitivities, food texture difficulties

X Symptoms present in early developmental period

X Symptoms cause clinically significant impairment in social, occupational, and other areas of functioning

X Disturbances not better explained by Intellectual Disability / Intellectual Developmental Disorder / Global Developmental Delay.

Specify if:

With accompanying intellectual impairment
With accompanying language impairment
With known medical / genetic condition or environmental factor: _____
With neurodevelopmental / mental / behavioral disorder: _____
With catatonia

EVALUATION TIMELINE

Professional Psychological Evaluation Service	Date Work Completed	Start Time	Stop Time	Total Units (CPT Code)
Record Review, Referral Review, Test Selection	1/31/2022	9:00	9:30	1 (96113)
Test Administration – WISC, ADOS	1/31/2022	10:00	12:00	1 (96112) 2 (96113)
Behavioral Screenings – ABAS, BASC	1/31/2022			2 (96127)
Interpretation of Test Results & Preparation of Recommendations	1/31/2022	12:00	1:00	2 (96113)
Face-to-Face Interpretive Feedback	1/31/2022	1:00	2:00	2 (96113)
Preparation of the Report	1/31/2022	2:00	3:30	3 (96113)

Total Time Spent: 6 hours
Total Units: 96112 (1 units)
96113 (10 units)

Behavioral screenings
96127 (2 units)